

Application of Social Learning Theory to Employee Self-Management of Attendance

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Training in self-management was given to 20 unionized state government employees to increase their attendance at the work site. Analyses of variance revealed that compared to a control condition ($n = 20$), training in self-regulatory skills taught employees how to manage personal and social obstacles to job attendance, and it raised their perceived self-efficacy that they could exercise influence over their behavior. Consequently, employee attendance was significantly higher in the training than in the control group. The higher the perceived self-efficacy, the better the subsequent job attendance. These data were significant at the .05 level.

Kanfer's (1970, 1975, 1980) training in self-management teaches people to assess problems, to set specific hard goals in relation to those problems, to monitor ways in which the environment facilitates or hinders goal attainment, and to identify and administer reinforcers for working toward, and punishers for failing to work toward, goal attainment. In essence, this training teaches people skills in self observation, to compare their behavior with goals that they set, and to administer reinforcers and punishers to bring about and sustain goal commitment (Karoly & Kanfer, 1982). The reinforcer or punisher is made contingent on the degree to which their behavior approximates the goal. Kanfer viewed these two outcome variables in terms of informational as well as emotional feedback in order to account for cognitive as well as motoric and autonomic effects. Essentially, however, this represents a broadening rather than a change of the reinforcement contingency concept.

Training in self-regulation has been evaluated rigorously in both laboratory and clinical settings. Positive results have been obtained with regard to teaching oneself to stop smoking (Kanfer & Phillips, 1970), to overcome drug addiction (Kanfer, 1974), to reduce weight (Mahoney, Moura, & Wade, 1973), to improve study habits (Richards, 1976), and to enhance academic achievement (Glynn, 1970).

One theory that explains the effectiveness of training in self-management is social learning theory (Bandura, 1977a; 1986). This theory emphasizes the role of self-reactive influences in motivating and guiding one's behavior. The theory states that by arranging environmental contingencies, establishing specific

goals, and producing consequences for their actions, people can be taught to exercise control over their behavior.

Two social learning theory constructs that may underlie the effectiveness of training in self-management are perceived self-efficacy and outcome expectancies. Perceived self-efficacy refers to the strength of one's belief that he or she can successfully execute the behaviors required (Bandura, 1982). Such self-beliefs influence what people choose to do, how much effort they mobilize, and how long they will persevere in the face of real or perceived obstacles. For example, people who judge themselves as inefficacious in coping with environmental demands may imagine their difficulties (e.g., family obligations, transportation issues) as more formidable than they are in fact. In contrast, people who have a strong sense of self-efficacy focus their attention and effort on the demands of the situation and are spurred to an increase in effort by perceived obstacles (Bandura, 1982).

Outcome expectancies refer to beliefs concerning the extent to which one's behavior will produce favorable or unfavorable outcomes. People are prone to act on their self-percepts of efficacy when they believe that their actions will produce outcomes that are beneficial to them (Bandura, 1982). However, they are unlikely to change their behavior when they believe they can perform competently, but that the environment (e.g., supervisory or peer evaluation) will be unresponsive to their improved performance.

Few, if any, empirically based experiments have been conducted in organizational settings on the efficacy of training in self-management. Because of the diversity of clinical problems in which this training has proven effective and because of the strong theoretical rationale on which this training is based, the present study investigated its effectiveness with regard to increasing employee attendance.

Low job attendance or absenteeism is a chronic problem in organizational settings (Goodman & Atkin, 1984). Recent estimates place the annual cost of absenteeism in the United States at approximately \$30 billion (Steers & Rhodes, 1984). This is because employee absences can disrupt work schedules, increase costs, and decrease productivity.

The implicit theory underlying the present study with regard

Preparation of this article was supported in part by the Ford Motor Company Fund. The article is based on the first author's doctoral dissertation completed at the University of Washington under the supervision of the second author.

We express our gratitude to Albert Bandura, Frederick Kanfer, Edwin Locke, and Terry Mitchell for their constructive comments on earlier drafts of this article, and to H. D. Beach for his advice on conducting the training.

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to job attendance is that many people judge themselves as inefficient in coping with environmental demands that prevent them from coming to work. Furthermore, they may believe that neither managers nor peers will change their low opinion of them even if they do increase their attendance. Support for the first assumption can be found in the revision of the Steers and Rhodes (1984) model. These authors introduced perceived inability to come to work as a critical variable affecting employee attendance. In addition, Chadwick-Jones, Nicholson, and Brown (1982) showed how cultural and normative variables can affect attendance negatively. Even a variable so straightforward as one's work schedule (flexible for most white collar jobs; inflexible for most blue collar jobs) can have a positive or negative effect on an employee's attendance. For example, people in a blue collar unionized job may find it extremely difficult to take an hour off in the middle of the workday to accompany a child to a medical doctor's office, whereas most people in white collar jobs can do this with relative ease. Thus, the former person may take the day off to accomplish what the latter person can do in 2 or 3 hr.

Support for the assumption regarding outcome expectancies is based on anecdotal evidence obtained in industry. Many people feel that once they have been labeled *poor employees* it is difficult for them to change their reputation. Support for this belief can be found in research on attitude perseverance in the face of contradictory evidence (Ross, 1977).

The primary hypothesis of the present study was that skill in self-management is a causal or independent variable that affects employee attendance positively. A second hypothesis was that the intervening variables are perceived self-efficacy and outcome expectancies. In addition to job attendance, reaction and learning measures were used as criteria for evaluating the effectiveness of the training program.

Method

Sample

A meeting was held among the first author, the director of personnel, and a representative from the union to explain how training in self-management would be a positive approach to increasing job attendance. The union agreed to support the training if certain conditions were agreed on by management, namely, that no employee would be required to receive the training, that no monetary incentive would be offered for increasing one's attendance, and that the course would be offered during normal work hours.

Employees who had used 50% or more of their sick leave received a memo from the personnel department inviting them to participate in this study. In an attempt to minimize attrition rates reported in other studies (e.g., Harris & Ream, 1978), the memo stressed that only persons who would commit themselves to eight 1-hr weekly group sessions and eight 30-min weekly one-on-one sessions should volunteer for the training. Furthermore, people who had scheduled vacation time should not enroll in the training at this time. Finally, people who were on disciplinary probation could not participate in the training.

Of the 50 individuals who were contacted, 42 volunteered to receive the training. Of these, 2 stated that they would not be able to attend all eight sessions because of scheduled vacation time. The remaining 40 individuals were randomly assigned to an experimental ($n = 20$) or a control group ($n = 20$). The people in the control group were told that they would be trained at a later date.

The mean age of the 40 employees was 44.33 years ($SD = 11.4$ years). Of the participants, 70% were men, and 30% were women. The mean number of years they had worked for the state was 7.41 ($SD = 3.14$). The individuals were employed in a maintenance department as carpenters, electricians, and painters. None of these people dropped out of the training program.

Procedure

The control group, like the experimental group, was exposed to ongoing organizational sanctions (e.g., 2 or more days off per month without a medical slip, failure to call in) regarding absenteeism. These sanctions consisted of an oral warning, a written warning, being placed on 3-months probation, and termination. The incentive for job attendance was that employees earned 8 hr sick leave each month. Hours that were not used by the end of the year would be applied to the next year. People were given compensation on retirement for the total number of sick leave hours that were not used. These policies had been in existence for 12 years. Nevertheless, the mean recorded absenteeism due to sick leave was 5.26 employee hr per week ($SD = 3.61$) in the experimental group and 4.96 employee hr in the control group ($SD = 2.16$).

The training program itself consisted of eight weekly 1-hr group sessions followed by eight 30-min one-on-one sessions. Each training group consisted of 10 people. The one-on-one sessions were conducted to tailor the training to the specific concerns of each individual and to discuss issues that the person might have been reluctant to introduce in a group setting.

The first week an orientation session was conducted to explain the principles of self-management. The second week, the reasons given by the trainees for using sick leave were listed and classified into nine categories, namely, legitimate illness, medical appointments, job stress, job boredom, difficulties with coworkers, alcohol and drug related issues, family problems, transportation difficulties, and employee rights (i.e., "sick leave belongs to me"). Of these nine categories, family problems, incompatibility with supervisor or coworkers, and transportation problems were listed most frequently. Sick leave was the focus of discussion in this session because it accounted for 49.8% of the recorded absenteeism in the organization.

The trainees were taught to develop a description of the problem behaviors (e.g., difficulty with supervisor), to identify conditions that elicited and maintained the problem behaviors, and to identify specific coping strategies. This constituted the session on self-assessment. In this session, as in all sessions, the employees were assured that their comments would not be shared with anyone outside the training group.

The third week focused on goal setting. The distal goal was to increase one's attendance within a specific time frame (e.g., 1 month/3 months). The proximal goals were the specific behaviors that the respective individual had to engage in to attain the distal goal.

The fourth week focused on the importance of self-monitoring one's behavior. Specifically, the trainees were taught (a) to record their own attendance, (b) the reason for missing a day of work, and (c) the steps that were followed to subsequently get to work. This was done through the use of charts and diaries. Emphasis was placed on the importance of daily feedback for motivational purposes as well as accuracy in recording.

In the fifth week the trainees identified reinforcers and punishers to self-administer as a result of achieving or failing to achieve the proximal goals. The training emphasized that the reinforcer must be powerful and easily self-administered (e.g., self-praise, purchasing a gift). The punisher was to be a disliked activity, easily self-administered (e.g., cleaning the garage). Each individual developed specific response-reward contingencies.

The sixth week was essentially a review of the previous six sessions. This was accomplished by asking the trainees to write a behavioral con-

tract with themselves. Thus, each trainee specified in writing the goal(s) to be achieved, the time frame for achieving the goal(s), the consequences for attaining or failing to attain the goal(s), and the behaviors necessary for attaining the goal(s).

The seventh week emphasized maintenance. Discussion focused on issues that might result in a relapse in absenteeism, planning for such situations should they occur, and developing coping strategies for dealing with these situations.

The theoretical rationale for combining these variables into one treatment package can be found in Bandura (1977b). The assumption underlying training in self-management is that the treatment package should "include as many component procedures as seem necessary to obtain, ideally, a total treatment success" (Azrin, 1977, p. 144). Empirical support for combining goal setting, feedback, and self-monitoring into a treatment package can be found in both the organizational behavior and clinical psychology literature. For example, Erez (1977) found that goal setting in the absence of feedback has no effect on behavior. Latham, Mitchell, and Dossett (1978) found that feedback in the absence of goal setting has no effect on behavior subsequent to a performance appraisal. Similarly, Simon (1979) showed that self-monitoring in the absence of goal setting has no effect on behavior. Campbell (1982) concluded that little would be gained from further attempts to tease apart the relative effects of goal setting, feedback, and reinforcers.

Results

Manipulation Checks

Goal commitment. The internal consistency of a four-item commitment measure (e.g., "To what extent will you strive to attain the goal?" "How important is it to you to at least attain the goal that was set?") administered during the final week of training was satisfactory (coefficient $\alpha = .81$). The mean of the responses to the 5-point Likert-type items was extremely high ($M = 4.73$, $SD = .22$). This restriction in range for uniformly high commitment precluded significant correlations between goal commitment and performance on the learning test, or with job attendance.

Application. A key concern was whether the trainees used the skills that were taught in the training class. Three months after the training program had been completed, the trainees were interviewed. Responses to a 5-point 13-item questionnaire on the extent to which goals were being set, feedback charts were being maintained, and reinforcers and punishers were being administered correlated significantly ($p < .05$) with job attendance ($r = .48, .45, .47$, respectively). Further corroboration that the trainees were applying the training content was obtained when the first author's visually inspected the feedback charts. Only 3 people were not keeping attendance charts systematically.

Criterion Measures

Reaction measures. Assessing employee reactions to the training was important because many trainees had argued that sick leave is a "privilege that belongs to me." Thus, it was important to determine what the trainees perceived was especially effective or ineffective about the program.

In the initial sessions, the trainees expressed hostile reactions to the training in the form of self-deprecating and aggressive comments (e.g., "I guess we are the delinquent bunch"; "the trainer is a spy for management"). A fist fight occurred in the first class as a result of name-calling between two trainees.

A 5-point five-item Likert-type questionnaire (e.g., "The training I received helped me overcome obstacles preventing me from coming to work") was completed anonymously immediately after and again 3 months after the training to measure employee reactions to the training. The coefficient alphas for this questionnaire were .70 and .73.

The employee reactions to the training were very positive immediately after training ($M = 4.32$, $SD = .55$). The employees expressed the same positive reaction 3 months after the training had taken place ($M = 4.46$, $SD = .41$). The test-retest reliability of this measure was .81.

Specifically, the trainees reported that the training enabled them to identify obstacles that prevented them from coming to work; it helped them overcome these obstacles; it led them to set specific goals for increasing job attendance; and it increased their confidence in their ability to control their own behavior.

Learning measure. It is important to understand in what way the training was effective. One criterion is learning (Kirkpatrick, 1976; Wexley & Latham, 1981). Did the trainees learn ways of responding to attendance related issues? Did they acquire problem-solving principles that enabled them to deal with coming to work effectively? To answer these questions, a learning test was developed on the basis of interviews with the supervisors of the employees.

The learning test consisted of 12 situational items with a scoring guide. A sample item is the following:

The reason I don't come to work is that I do not get along with a particular person with whom I work. Whenever she is on shift, I call in sick. I noted that when I do have contact with her on the job, we get into arguments. I decided to set a goal of "getting along with her", but it does not seem to be working. What should I do?

This methodology is based on the situational interview (Latham & Saari, 1984; Latham, Saari, Pursell, & Campion, 1980). The test was administered and scored prior to training (coefficient $\alpha = .74$) and again 3 months after the training (coefficient $\alpha = .82$) by two judges who were blind to whether the responses were from people in the training or control group. The test-retest reliability of responses to the items was .85.

An analysis of variance (ANOVA) revealed no significant difference between groups prior to training. However, the difference between groups was highly significant subsequent to training, $F(1, 38) = 6.30$, $p < .02$, $\eta^2 = .10$. The mean of the training group's performance on this test was 29.95 ($SD = 7.0$); the mean of the control group was 16.4 ($SD = 2.9$). The correlations between the premeasure and the postmeasure with job attendance were .35 ($p < .05$) and .77 ($p < .05$), respectively.

Attendance. Employee absenteeism was defined by the organization as falling into 1 of 11 categories (e.g., holiday leave, sick leave, vacation leave, jury leave, bereavement leave). Because 49.8% of the absenteeism was recorded as sick leave, this measure was used in this study. This measure was operationally defined as the number of sick leave hours taken per employee each week. Attendance was defined as the number of hours on the job when the employee was scheduled to be at work. The total number of hours the person could work on the job each week was 40. Overtime was not permitted by the organization. These two measures permitted multiple operationalism of the dependent variable of primary interest, namely, employee presence at the work site.

The test-retest reliability (stability) of the recording of sick leave assessed over a 52-week period prior to conducting the study was .38. This was in sharp contrast to the reliability of the weekly measures of attendance, namely .90. The test-retest reliability of the recording of sick leave assessed over the 12 weeks subsequent to this study was .42; the test-retest reliability of the attendance measure was .92. The correlation between the 12-week measure of sick leave and attendance was $-.64$.

A repeated measures multivariate analysis of variance based on 12 weeks of data on these two dependent variables revealed a significant difference between the training and the control group, $T(2, 37) = 6.67, p < .05$. Univariate F tests revealed a significant difference between the training group ($M = 458.4, SD = 32.7$) and the control group ($M = 403.2, SD = 22.8$) for the measure of attendance, $F(1, 38) = 5.52, p < .05, w^2 = .10$. However, the F test was only marginally significant, $F(1, 38) = 1.84, p < .10, w^2 = .027$, for the measure of sick leave ($M = 69.6, SD = 12.3; M = 83.1, SD = 18.5$, training and control group, respectively). This latter finding undoubtedly reflects the lack of stability in this absenteeism measure due to criterion contamination relative to the measure of job attendance. The accuracy of the measure of sick leave was dependent on self-report.

Intervening Variables

Of critical importance to this research was understanding why the training was effective from a psychological standpoint. Did training in self-management affect one's perceived self-efficacy and outcome expectancies? Do these variables predict job attendance?

Perceived self-efficacy and outcome expectancies were measured prior to the study, immediately after the study, and again 3 months later. The 15-item perceived self-efficacy scale followed the format used to measure self-efficacy with regard to refraining from smoking (Conditte & Lichenstein, 1981). Other items were based on comments received from supervisors ($n = 12$) and employees ($n = 10$) regarding obstacles affecting a person's coming to work. For each of the 15 items, the trainees indicated whether they felt that they would be able to come to work in each of the job attendance situations described (efficacy level) and, if yes, rated their confidence separately on a scale from 0 to 100 (efficacy strength). The coefficient alphas were .88, .91, and .89. The test-retest reliability between Time 1 and Time 3 was .92, and between Time 2 and Time 3 was .94.

Outcome expectancies (e.g., "I will not be able to meet family demands"; "I will increase my sense of accomplishment") were measured using a 15-item questionnaire that contained both positive and negative consequences, as perceived by the employee, for coming to work. These items were generated from informal interviews with supervisors and personnel officers prior to conducting the study. For each item, individuals were asked to designate on a 100-point probability scale (expressed in % units), ranging in 10-unit intervals, the probability that they would experience or achieve a particular outcome as a result of coming to work. The coefficient alphas were .67, .63, and .68, respectively. The test-retest reliability based on Time 1 and Time 3 was .74, and between Time 2 and Time 3 was .76.

There was no significant difference between the training and

control group on the measure of perceived self-efficacy or outcome expectancies prior to the training. A 2×3 repeated measures ANOVA for the self-efficacy scores revealed a significant F for groups, $F(1, 119) = 24.78, p < .05, w^2 = .24$; time, $F(2, 119) = 16.71, p < .05, w^2 = .16$; and Group $\times$ Time interaction, $F(2, 119) = 46.02, p < .05, w^2 = .26$. The experimental group ($M = 102.3, SD = 17.5$) not only expressed higher self-efficacy than did the control group ($M = 81.1, SD = 12.4$), but this difference in perceived self-efficacy increased over time. A Pearson r between the strength of the self-efficacy measure taken immediately after training and subsequent job attendance as well as sick-leave was significant ($r = .49, r = -.40, p < .05$, respectively).

The analysis with regard to outcome expectancies did not yield any statistically significant findings. This may be because outcome expectancies were uniformly high prior to conducting the study.

Discussion

The theoretical significance of this study is that it provides an explanation of why people do or do not come to work. As As (1962) and Fichman (1984) have noted, absenteeism has been a social fact in need of a theory. Self-efficacy (Bandura, 1977a, 1982) is one such theory. People who come to work may be individuals who are able to overcome the personal obstacles, as well as the cultural and group norms, that were identified by Chadwick-Jones et al. (1982) as affecting one's perceived ability to come to work. People who do not come to work may be unable to cope with these influences unless their efficacy is enhanced by providing them the skill to exercise control over these variables.

Of further theoretical significance was the finding that high outcome expectancies alone will not result in employees coming to work if they judge themselves as inefficacious in overcoming personal and social obstacles to work attendance. This finding is in accord with other studies that show that low perceived self-efficacy negates the motivating potential of outcome expectancies (Barling & Abel, 1983; Godding & Glasgow, 1985; Williams & Watson, 1985).

Outcome expectancies are usually measured in terms of perceived external rewards and punishers. Social learning theory also emphasizes the role of affective self-evaluative outcomes in self-regulation through internal standards or goals. When performance falls short of the goal a person seeks to achieve, self-dissatisfaction occurs that motivates increased effort (Bandura, 1986; Bandura & Cervone, 1983). Self-motivation is regulated by both perceived self-efficacy and self-evaluation. It would be informative in future research of employees in different occupations in different industries to measure self-evaluative outcomes as well as anticipated external ones.

The practical significance of this study is fivefold. First, it showed the external validity of training in self-management for unionized workers employed by a state government. Until the present study, training in self-management had been restricted primarily to people in clinical or educational settings. Reaction, learning, self-efficacy, and job attendance measures taken 3 months after the training showed that skill in self-management brings about a relatively permanent change in cognition and affect, in addition to behavior.

Second, it showed the effectiveness of training in self-management on a dependent variable that had not been previously studied using this technique—namely, employee attendance. Employee attendance, as noted earlier in the article, has significant cost implications for organizations.

Third, employee attendance at work increased on the basis of a straightforward 12-hr training program. The concepts of goal setting and reinforcers are well-known to most trainers. What is unique to this training is the emphasis on trainees developing a contract with themselves, in addition to self-administering reinforcers and punishers to facilitate goal commitment.

Fourth, this study provided a stringent test of training in self-management. The control group, like the experimental group, was not only exposed to organizational rewards and penalties regarding attendance and absenteeism, but they had the desire to increase their attendance at work. Evidence for the latter is indicated by their attendance at the orientation session in which people were randomly assigned to the experimental or control groups. Thus, the rival hypothesis that the effects of this training were due to evaluation apprehension or attention was rejected.

Fifth, the study showed the importance of using attendance rather than a measure of absenteeism as the primary dependent variable. This point has been argued elsewhere (Latham & Frayne, 1986; Latham & Napier, 1984), but the superiority of the former measure for assessing the effects of an intervention had not been demonstrated empirically. Measures of absenteeism are typically nothing more than measures of the categorization behavior of recorders (Latham & Pursell, 1975, 1977). They typically reflect the outcome of negotiated behavior between a superior and a subordinate. That is, an absence is sometimes classified as sick leave rather than as a vacation day as a reward for good performance (Goodman & Atkin, 1984). Thus, absenteeism measures are highly contaminated (Thorndike, 1949), and their reliability is typically quite low. Had only a measure of recorded sick leave been used in this study as the sole index of absenteeism, a Type II error would have been made. The results would have shown that training in self-management had only a marginal effect on employee absenteeism.

A limitation of both the attendance and the absenteeism measures is that they ignore the distinction between voluntary and involuntary absenteeism. Some people may take sick leave because they are too ill to come to work, whereas others may have negotiated with a supervisor to record a vacation day as sick leave. These two behaviors, illness and negotiation, are very different theoretically. This lack of sensitivity in the two measures provided a highly conservative test of the training program. To overcome this problem in future studies of absenteeism, each person who stayed away from work would have to be observed, and the interobserver reliability of the observation would have to be calculated. To the authors' knowledge, this has never been done in the research literature on absenteeism.

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Received May 13, 1986

Revision received January 23, 1987

Accepted January 29, 1987 ■